Daily Life and Wellness: How Hispanic Cultural Origins Shape Health Behaviors

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Abstract

This literature review synthesizes ten peer-reviewed empirical studies (2025) examining how cultural factors shape health behaviors among Hispanic/Latino populations in the United States across smoking cessation, physical activity, diabetes prevention, dementia caregiving, obesity, mental health, and substance use. Cultural factors operate at multiple levels: language preference affects healthcare navigation; familismo (family orientation) functions dually as support and stressor; acculturation paradoxically increases discrimination exposure among US-born individuals; health literacy mediates preventive behaviors (62% variance explained); and discrimination impacts mental health outcomes. Spanish-language, culturally adapted physical activity interventions achieved exceptional retention rates (90-100%), demonstrating that cultural adaptation enhances engagement. Effective interventions must recognize within-group heterogeneity, prioritize health literacy, and address structural barriers.

Keywords: Hispanic health, acculturation, health literacy, health disparities, cultural adaptation, preventive behaviors, mental health

Introduction

The Hispanic/Latino population in the United States represents a diverse group with varying cultural backgrounds, migration histories, and health outcomes. Understanding how cultural origins shape health behaviors is essential for developing effective, culturally tailored interventions that address health disparities in this growing population. While much research has documented health disparities affecting Hispanic communities, less attention has been paid to how cultural origins shape daily health behaviors and wellness practices. This review examines cultural factors including familismo (family orientation), acculturation levels, language preferences, health literacy, and ethnic identity.

This literature review examines recent empirical research (2025) investigating the relationship between Hispanic cultural origins and health behaviors across multiple domains. The ten studies reviewed include participants from diverse Hispanic subgroups: Mexican and Mexican American (the largest represented group), Puerto Rican, Cuban, Dominican, Central American, and South American populations. Participants were predominantly first- and second-generation immigrants, though samples also included third-generation and U.S.-born individuals. Geographic settings spanned U.S. urban areas including New York City, Los Angeles, Chicago, and communities across California, Texas, Washington, Oregon, Utah, and national online samples.

The health behaviors examined include smoking cessation, physical activity, diabetes prevention and management, obesity, dementia caregiving, traumatic brain injury recovery, mental health outcomes (anxiety, depression, loneliness), discrimination experiences, and substance use among youth. Cultural factors analyzed include familismo dynamics, language preference, acculturation and acculturative stress, health literacy, cultural beliefs about personal responsibility, and experiences of discrimination and microaggressions. Table 1 provides an overview of the included studies.

Table 1 *Overview of Included Studies*

Overview of Included Studies					
Study	**Design**	**Sample Size**	**Population**	**Health Domain**	**Key Findings**
Correa-Fernandez et al. (2025)	Qualitative (SEM)	18	Spanish speaking Latinx adults with Psychological	Smoking cessation	Familismo operates dually: family support helpful but family demands create stress

Overview of Included Studies					
			distress		
Estrada et al. (2025)	Qualitative Interviews	19	Latino dementia caregivers, NYC	Dementia caregiving	Caregivers experience isolation when family doesn't share responsibilities contrary to cultural expectations
Garcia et al. (2025)	Qualitative (CFIR)	12	Hispanic TBI program implementers	Traumatic brain injury navigation	Bilingual/bicultural navigators essential; immigration concerns major barrier
Gonzalez et al. (2025)	Qualitative interviews	15	Hispanic men with prediabetes	Diabetes prevention	Surprise at diabetes risk despite family history; work environment and cultural foods create barriers
Loya et al. (2025)	Pre-post pilot intervention	21 (19 completed)	Hispanic adults with Type 2 Diabetes	Physical activity	Steps nearly doubled (23,006 → 43,001, p=.005); 90.5% retention during COVID-19
Odame et al. (2025)	Cross-sectional national survey	1,126	Hispanic/Latino adults (55.51% US-born)	Mental health & discrimination	38.19% experienced daily/weekly discrimination; US-born 30% more likely than foreign born; anxiety/depression increases risk 138%
Sanchez-Johnsen et al. (2025)	CBPR baseline	199	Latina women (Mexican, Puerto Rican)	Obesity	Significant within-group differences: Mexican women younger, less educated, higher Spanish preference, lower obesity than Puerto Rican women
Tailakh et al. (2025)	Cross-sectional path analysis	149	Hispanic adults, Southern California	Health literacy & preventative behaviors	Healthy literacy explained 62% variance; education strongest predictor (β = .74); health literacy → adherence (β=.51)
Villamil Grest et al. (2025)	Longitudinal LCA (3 timepoints, 2005 - 2016)	1,179	Latino/Hispanic youth in Los Angeles	Substance use & ACEs	4 adversity classes identified: "Psychological abuse & discrimination" class showed higher alcohol use (B=.316, p=.04)
Zlatar et al. (2025)	Pre-post pilot intervention	10	Hispanic older adults (mean age 62.7)	Physical activity	100% completion; high satisfaction (4.6/5); barriers: low self-efficacy, time; facilitators: social support, Fitbit

Note. SEM = Social Ecological Model; CFIR = Consolidated Framework for Implementation Research; TBI = Traumatic Brain Injury; CBPR = Community-Based Participatory Research; LCA = Latent Class Analysis; ACEs = Adverse Childhood Experiences.

Source. Reviewed literature (2025).

Figure 1. How Hispanic Cultural Origins Shape Health Behaviors

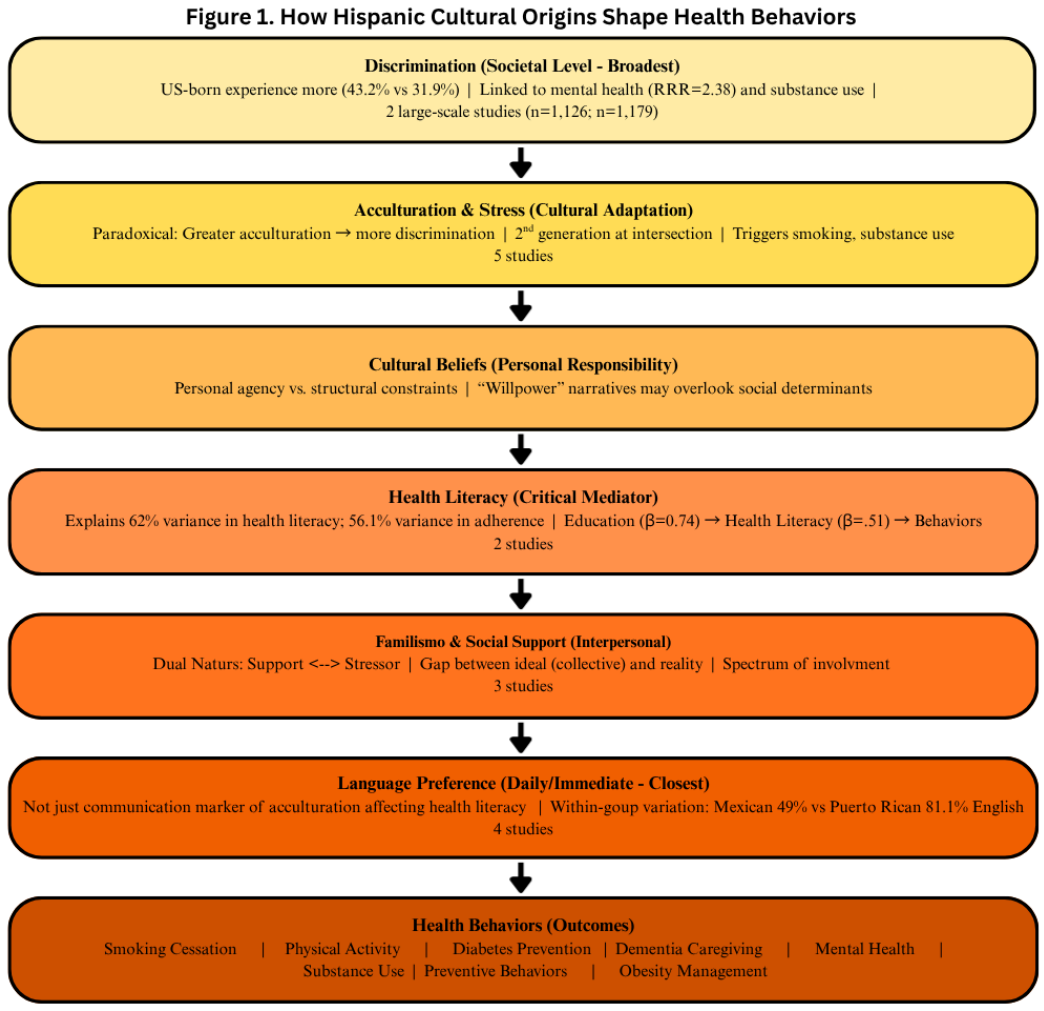

Figure 1. Multilevel framework illustrating how Hispanic cultural origins shape health behaviors across six levels, from language preference (most immediate) to discrimination (broadest societal). Health literacy (middle level, highlighted) emerges as a critical mediator explaining 62% of variance in preventive behaviors. *Source.* Author's conceptual synthesis of reviewed studies.

Methodology

This review includes ten peer-reviewed empirical studies published in 2025 that examined Hispanic/Latino health behaviors through cultural lenses. Studies were identified through systematic searches on Embase, PsycINFO, and ProQuest Central databases conducted in January 2026. Search terms included combinations of: ("Hispanic" OR "Latino" OR "Latina" OR "Latinx" OR "Mexican American" OR "Puerto Rican") AND ("health behavior*" OR "health outcome*" OR "preventive behavior*") AND ("cultural factor*" OR "familismo" OR "acculturation" OR "health literacy" OR "language preference") AND ("2025"). Additional studies were identified through citation tracking.

Studies were selected based on: (1) peer-reviewed journal articles with original empirical data, (2) focus on Hispanic/Latino populations in the United States, (3) examination of cultural factors influencing health behaviors, (4) published in 2025, (5) empirical research designs (qualitative, quantitative, or mixed methods). Excluded were literature reviews (n=3), theoretical papers without empirical data (n=2), and conference presentations not yet peer-reviewed (n=1).

The ten studies demonstrated diverse methodological approaches: four qualitative studies (Correa-Fernández et al., Estrada et al., Garcia et al., Gonzalez et al.), three quantitative cross-sectional studies (Sánchez-Johnsen et al., Tailakh et al., Odame et al.), one longitudinal study (Villamil Grest et al.), and two pilot intervention studies (Loya et al., Zlatar et al.). Sample sizes ranged from 10 to 1,179 participants.

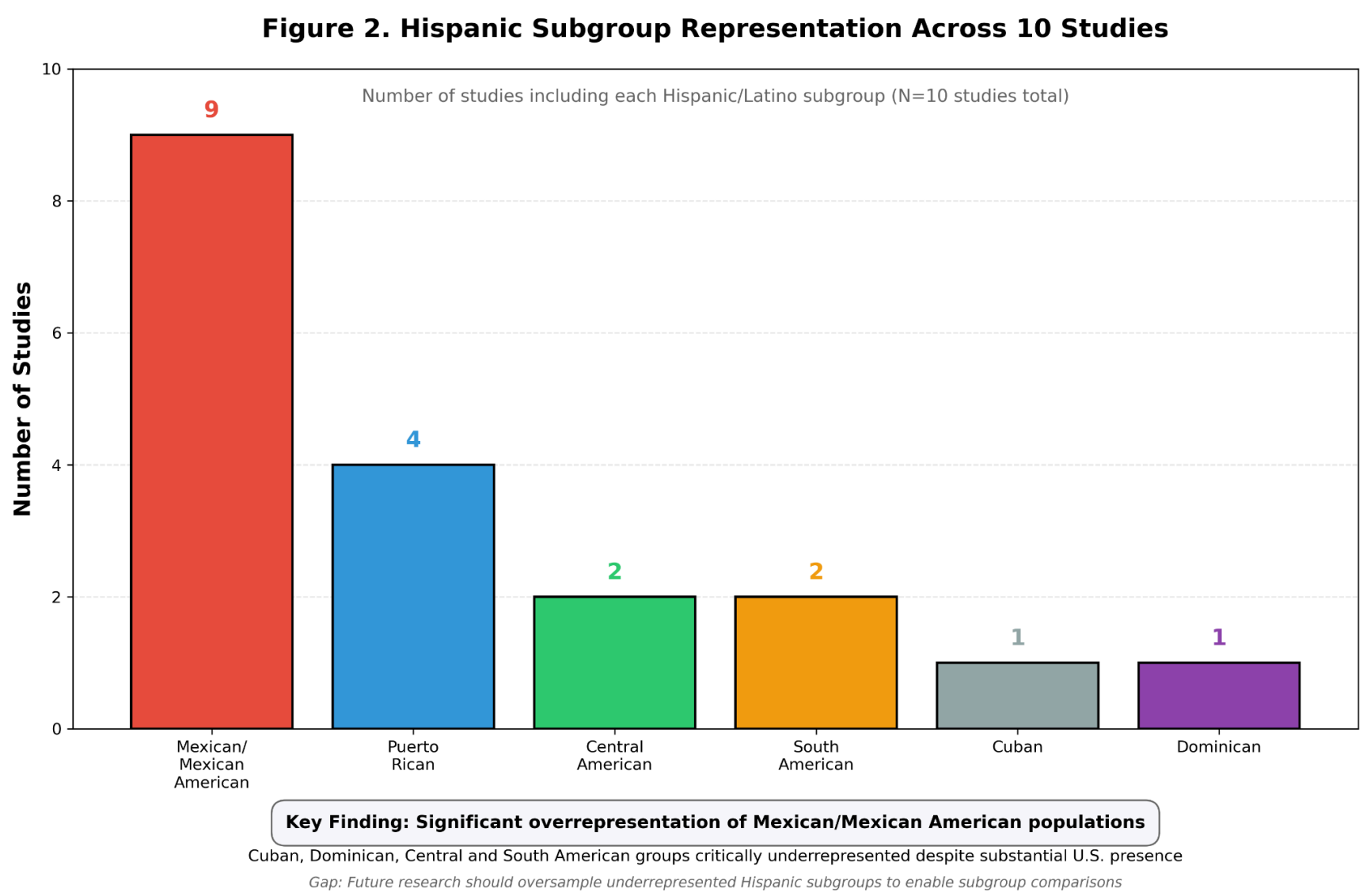

Figure 2. Hispanic subgroup representation across 10 studies (N=10). Mexican/Mexican American populations appear in 9 of 10 studies; Cuban and Dominican populations appear in only 1 study each. *Source.* Author's analysis of reviewed studies.

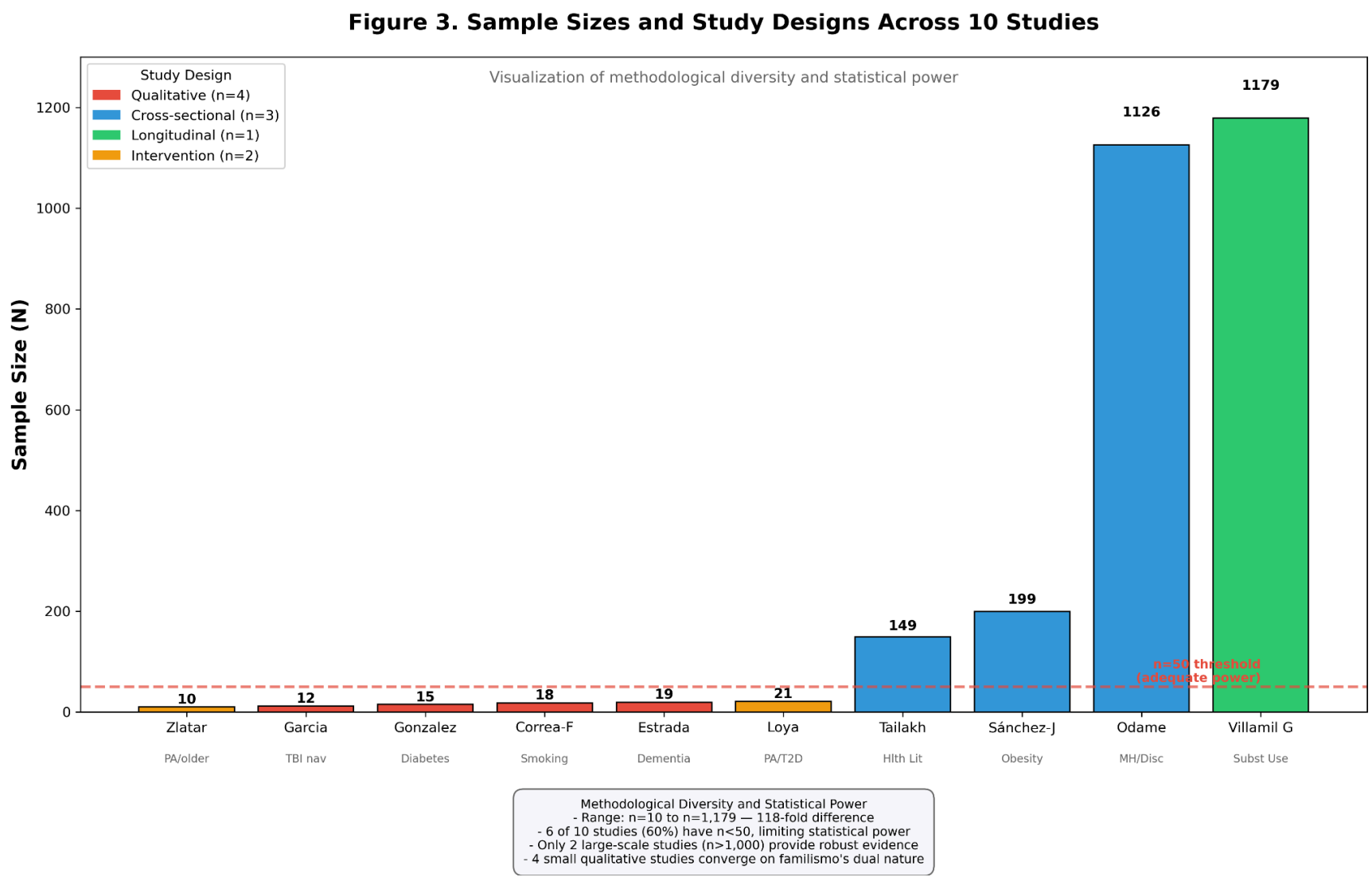

Figure 3. Sample sizes and study designs across 10 studies, illustrating methodological diversity and substantial variation in statistical power. Sample sizes range from n=10 to n=1,179. *Source.* Author's analysis of reviewed studies.

Figure 3 illustrates the substantial variation in statistical power across the ten studies, with sample sizes ranging from n=10 to n=1,179 a 118-fold difference, and confirms that only two large-scale studies (n>1,000) provide robust statistical evidence, while six studies (60%) have samples below n=50.

The Role of Cultural Factors in Health Behaviors: Cross-Cutting Themes

The following sections examine cultural factors organized from the most immediate daily experiences (language use) outward through increasingly broader contexts (family, cultural adaptation, individual capacities, and societal forces). This framework illustrates how daily experiences expand through family relationships and individual capacities to broader societal forces.

Language Preference and Healthcare Access

Language emerged as the most immediate cultural factor influencing health behaviors and healthcare navigation across four studies (Table 2). Language preferences varied substantially by Hispanic subgroups, challenging assumptions of linguistic homogeneity (all being the same). Sánchez-Johnsen et al. (2025) found Mexican women showed significantly higher Spanish preference than Puerto Rican women (49% vs 81.1% English preference, $p<.001$), demonstrating that interventions cannot assume language needs based solely on ethnicity. Garcia et al. (2025) identified bilingual/bicultural navigators as essential facilitators, though families' immigration concerns created hesitancy to seek assistance, suggesting language barriers intersect with broader structural vulnerabilities. Tailakh et al. (2025) revealed that speaking English at home predicted higher health literacy ($\beta=.25$, $p<.05$), suggesting language operates as a marker of broader acculturation processes affecting health knowledge.

Table 2 *Language Preference and Healthcare Access Across Four Studies*

Study	N	Population	Key Language Findings	Implication
Garcia et al. (2025)	12	TBI program implementers	66.7% Spanish-speaking navigators essential; immigration concerns create hesitancy	Bilingual navigators are facilitators; language intersects with structural vulnerabilities
Sánchez-Johnsen et al. (2025)	199	Latina women (Mexican, Puerto Rican)	Mexican women: 49% English preference; Puerto Rican: 81.1% English preference ($p<.001$)	Cannot assume language homogeneity within Hispanic populations
Correa-Fernández et al. (2025)	18	Spanish-preferring smokers	73.7% preferred Spanish for daily communication; strong preference for Spanish interventions	Spanish-language services remain critical for U.S. residents
Tailakh et al. (2025)	149	Hispanic adults	Speaking English at home predicted higher health literacy ($\beta=.25$, $p<..05$)	Language is marker of acculturation affecting health knowledge

Note. All four studies demonstrate language is not merely a communication barrier but intersects with acculturation, health literacy, and healthcare access. Within-group variation is substantial.

Source. Garcia et al. (2025), Sánchez-Johnsen et al. (2025), Correa-Fernández et al. (2025), and Tailakh et al. (2025).

Familismo and Social Support

Familismo emerged across three qualitative studies as operating dually rather than uniformly (Table 3). Correa-Fernández et al. (2025) found that while family support facilitated smoking cessation, family demands simultaneously triggered smoking among Spanish-speaking Latinx adults. Participants described a spectrum of desired family involvement, suggesting interventions should be flexible rather than prescriptive. Estrada et al. (2025) identified profound caregiver isolation when families didn't share dementia care responsibilities, directly contradicting cultural expectations of collective caregiving. This gap between cultural ideals (familismo as shared responsibility) and lived reality (isolated primary caregivers) created additional stress. Across studies, familismo operated complexly, neither universally positive nor negative, requiring interventions to assess rather than assume family dynamics and desired involvement levels.

Table 3 *Familismo as Support and Stressor Across Three Studies*

Study	N	Health Domain	Familismo as Support	Familismo as Stressor	Key
Correa-Fernández et al. (2025)	18	Smoking cessation	Family support helpful for quitting	Family demands create stress the triggers smoking	Dual role; spectrum of desired involvement
Estrada et al. (2025)	19	Dementia caregiving	Cultural expectation of collective caregiving	Isolation when family doesn’t share responsibility; gap between ideal and reality	Lack of support contradicts cultural expectations
Zlatar et al. (2025)	10	Physical activity	Family relationships important for sustain PA; social connection emphasized by CAB	Intervention focused on individuals, not family units	Family involvement valued but implementation varies

Note. PA = Physical Activity; CAB = Community Advisory Board. Convergent finding across all three studies: Familismo operates complexly, neither universally positive nor negative. Interventions must assess (not assume) family dynamics and desired involvement levels.

Source. Correa-Fernández et al. (2025), Estrada et al. (2025), and Zlatar et al. (2025).

Acculturation and Acculturative Stress

Acculturation emerged as both protective and risk factor across multiple studies (Table 4). Correa-Fernández et al. (2025) identified acculturation stress from work demands, financial pressures, and cultural adaptation as major smoking triggers among Spanish-speaking Latinx adults. Additionally, Odame et al. (2025) found US-born Hispanic/Latino individuals experienced more discrimination than foreign-born counterparts (43.20% vs 31.94%, $p<.001$), suggesting greater acculturation may increase discrimination exposure. Villamil Grest et al.'s (2025) sample (73.88% second-generation) positioned participants at the intersection of two cultures, potentially experiencing heightened acculturative stress. Both physical activity interventions recognized acculturation level influences engagement, with materials delivered entirely in Spanish at 5th-grade reading level for low-acculturated participants.

Table 4 *Acculturation as Protective and Risk Factor Across Five Studies*

Study	N	Design	Acculturation Measure/Indicator	Key Finding	Protective or Risk
Correa-Fernández et al. (2025)	18	Qualitative	Acculturative stress (work, finances, adaptation)	Triggers smoking behavior	Risk
Odame et al. (2025)	1,126	Cross-sectional	Birthplace (US-born vs. foreign-born)	US-born 30% more likely to experience discrimination ($p<.001$)	Risk (paradoxical)
Villamil Grest et al. (2025)	1,179	Longitudinal	Generation status (73.88% second generation)	Second generation at intersection of two cultures; heightened stress	Risk
Loya et al. (2025)	21	Intervention	SASH score (mean 1.53/5.0 = very Hispanic)	Low acculturation; 100% Spanish at home shaped intervention design	Context for intervention
Zlatar et al. (2025)	10	Intervention	Language (90% Spanish-speaking)	Low acculturation context; culturally adapted intervention	Context for intervention

Note. SASH = Short Acculturation Scale for Hispanics. Pattern: Acculturation operates as both protective and risk factor, but across these studies, evidence suggests greater acculturation (US birth, higher generation) paradoxically increases discrimination exposure and acculturative stress. Low-acculturated populations require Spanish-language, culturally grounded interventions.

Source. Correa-Fernández et al. (2025), Odame et al. (2025), Villamil Grest et al. (2025), Loya et al. (2025), and Zlatar et al. (2025).

Health Literacy as a Critical Mediator

Health literacy emerged as a crucial mediator between cultural factors and health behaviors (Figure 4, Table 5). Tailakh et al.'s (2025) path analysis explained 62% of variance in health literacy and 56.1% in adherence to preventive behaviors among 149 Hispanic adults. Education was the strongest predictor ($\beta=.74$), which strongly predicted adherence ($\beta=.51$), establishing health literacy as a key mechanism through which structural factors (education access) and cultural factors (language) influence health outcomes. Gonzalez et al. (2025) found 33.3% of Hispanic men with prediabetes lacked high school completion. These men expressed surprise at diabetes risk despite family history, suggesting knowledge gaps that health literacy interventions could address. The disconnect between family health history and personal risk perception indicates health literacy deficits prevent individuals from translating family health patterns into preventive action.

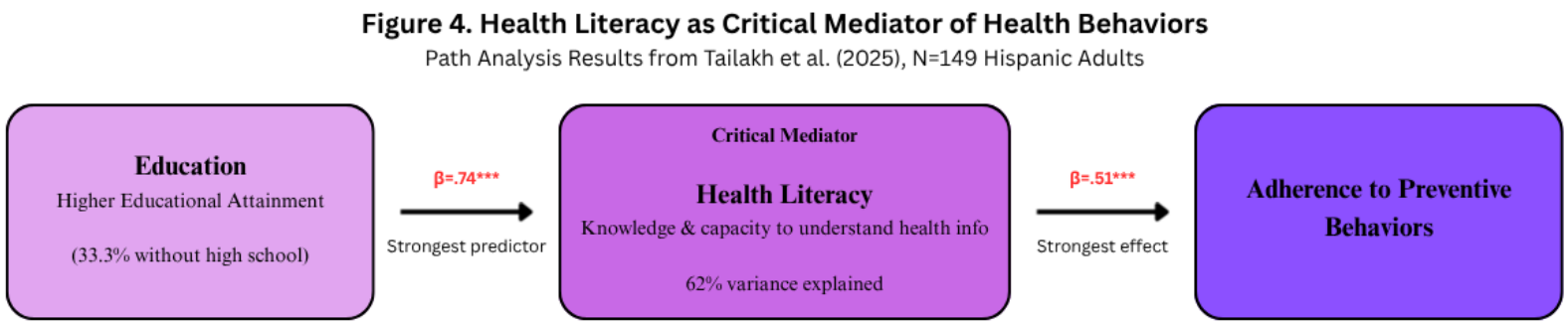

Figure 4. Path analysis results demonstrate health literacy as a critical mediator between education and adherence to preventive behaviors among 149 Hispanic adults in Southern California (Tailakh et al., 2025). Education is the strongest predictor of health literacy (β=.74, p<.001), explaining 62% of variance. *Source.* Adapted from Tailakh et al. (2025).

Path analysis results from Tailakh et al. (2025) demonstrate that health literacy operates as a critical mediator between education and adherence to preventive behaviors among 149 Hispanic adults in Southern California. Education is the strongest predictor of health literacy (β=.74, p<.001), explaining 62% of variance in health literacy. Health literacy, in turn, strongly predicts adherence to preventive health behaviors (β=.51, p<.001), explaining 56.1% of variance in adherence.

Table 5 *Health Literacy as Mediator of Preventive Health Behaviors*

Study	**N**	**Design**	**Health Literacy Measure**	**Key Findings**	**Mediating Pathway**
Tailakh et al. (2025)	149	Cross-sectional path analysis	Path analysis model	62% variance in health literacy explained; 56.1% variance in adherence explained)	Education → Health literacy (β=.74) → Adherence (β=.51)
Gonzalez et al. (2025)	15	Qualitative	Educational attainment (33.3% < high school)	Surprise at prediabetes risk despite family history; knowledge gaps)	Health literacy deficits prevent translating family history into preventative action

Note. Health literacy operates as critical mediator between structural factors (education access) and health behaviors. Educational gaps constrain health literacy, which in turn limits preventive health behaviors.

Source. Synthesis of Tailakh et al. (2025) and Gonzalez et al. (2025).

Cultural Beliefs About Personal Responsibility

Cultural beliefs about personal responsibility emerged as shaping health behaviors and treatment-seeking. Gonzalez et al. (2025) found Hispanic men with prediabetes (n=15) viewed lifestyle change as manageable and emphasized personal agency as necessary for change, yet simultaneously noted that restrictive work environments, overwhelming schedules, and carbohydrate-rich cultural food preferences created substantial barriers. This tension between beliefs about individual control and recognition of structural constraints reflects complex understanding of health behavior determinants. Similarly, Correa-Fernández et al. (2025) found Spanish-speaking Latinx smokers strongly endorsed beliefs about personal responsibility and "willpower" as necessary for quitting, even while acknowledging psychological distress, addiction, and social/environmental triggers. This emphasis on personal agency may reflect cultural values of self-determination (personalismo) but could indicate internalization of individual-responsibility narratives that overlook structural determinants of health.

Discrimination and Mental Health Disparities

Discrimination emerged across two large-scale quantitative studies as a fundamental stressor affecting Hispanic/Latino mental health and substance use (Table 6). Odame et al. (2025) found 38.19% of 1,126 adults experienced daily/weekly discrimination during COVID-19. Individuals with anxiety or depression were 138% more likely to report experiencing discrimination (RRR=2.38, 95% CI 1.54-3.70), with this association limited to US-born individuals (RRR=3.69). Villamil Grest et al. (2025) found youth experiencing combined psychological abuse and discrimination showed significantly higher alcohol use (B=.316, p=.04) than those experiencing microaggressions alone, suggesting trauma and discrimination should be addressed simultaneously in prevention efforts. This longitudinal evidence demonstrates that discrimination's effects accumulate over developmental periods.

Table 6 *Discrimination and Mental Health Across Two Large-Scale Studies*

Study	N	Design	Discrimination	Key Finding	Nativity/Generation Differences
Odame et al. (2025)	1,126	Cross-sectional national survey	Daily/weekly discrimination frequency	38.19% experienced discrimination daily/weekly; anxiety/depression RRR-2.38; loneliness RRR=1.29	US-born: 43.20%; Foreign-born:31.94% (p<0.001). Anxiety/depression predicted discrimination only for US-born (RRR=3.69)
Villamil Grest et al. (2025)	1,179	Longitudinal LCA (3 timepoints)	Latent class analysis of adverse experiences	"Psychological abuse & discrimination" class (31%) showed higher alcohol use (B=.316, p=0.04)	73.88% second generation (at intersection of cultures)

Note. Discrimination operates as a fundamental societal-level stressor affecting mental health and substance use. US-born/higher generation individuals experience more discrimination. Discrimination combined with psychological abuse is particularly harmful.

Source. Synthesis of Odame et al. (2025) and Villamil Grest et al. (2025).

Key Findings in Health Behavior Domains

While the previous section examined cultural factors that cut across health domains, this section explores how these factors manifest in specific health behaviors, revealing patterns in intervention preferences, implementation barriers, and population-level heterogeneity.

Physical Activity Interventions: Culturally Adapted Approaches

Two pilot studies achieved exceptional retention (Loya: 90.5%; Zlatar: 100%) through Spanish-language, culturally adapted approaches (Table 7). Loya et al.'s (2025) 6-week group intervention nearly doubled steps (23,006→43,001, p=.005) and physical activity (PA) minutes (106→224, p<.001) among adults with Type 2 Diabetes. Participants showed high baseline self-efficacy (3.53/4.0) but lacked structured opportunity and social support, which the intervention provided. Zlatar et al.'s (2025) 3-week remote intervention for older adults revealed high satisfaction (4.6/5) with requests for more group activities, suggesting optimal approaches combine remote accessibility with social elements. Both studies demonstrate Spanish-language, culturally grounded interventions are essential, not optional, for low-acculturated populations.

Table 7 *Comparison of Two Culturally Adapted Physical Activity Interventions*

Feature	Loya et al. (2025)	Zlatar et al. (2025)	Comparison
Population	21 adults with Type 2 Diabetes	10 older adults (mean age 62.7)	Different chronic conditions and age groups

Demographics	90% female, 95% Mexican, mean age 53	90% female, 90% Spanish-speaking, 90% Mexican/Mexican American	Both predominantly female, Mexican/Mexican American, Spanish-speaking
Acculturation	Very low (SASH=1.53/5.0); 100% Spanish at home	90% Spanish-speaking	Both low-acculturated populations
Duration	6 weeks	3 weeks	Loya longer
Format	Group-based	Remote/individualized	Different delivery models
Language	Spanish (5th grade reading level)	Spanish	Both Spanish-language
Development	Community-engaged	Community advisory board (Barrera & Castro framework)	Both culturally adapted through community input
Retention	90.5% (19/21)	100% (10/10)	Both exceptional; far exceed typical PA interventions
Primary Outcomes	Steps doubled (23,006 → 43,001, p=.005); PA minutes doubled (106 → 224, p<.001)	Qualitative barriers/facilitators; Satisfaction 4.6/5	Loya quantitative PA changes Zlatar feasibility/acceptabillity
Self-efficacy	High at baseline (3.53/4.0), remained stable	Identified low self-efficacy as barrier	Loya participants had confidence but lacked opportunity
Key Findings	Participants had confidence but needed structured opportunity and social support	Participants requested more group activities and staff interaction	Both suggest combining remote accessibility with group/social elements

Note. PA = Physical Activity; SASH = Short Acculturation Scale for Hispanics; CAB = Community Advisory Board. Cultural adaptation enhances retention; Spanish- language delivery is essential, not optional; low-acculturated populations successfully engage with culturally grounded interventions; optimal approach may combine remote accessibility with group format.

Source. Analysis of Loya et al. (2025) and Zlatar et al. (2025).

Intervention Preferences and Structural Barriers

Three studies revealed consistent patterns in intervention preferences and structural barriers across different health behaviors. Correa-Fernández et al. (2025) found Spanish-preferring Latinx smokers (n=18) expressed preferences for combination individual/group formats, in-person delivery, Spanish language, and incorporation of cultural elements like music and art, paralleling the physical activity interventions' emphasis on culturally grounded approaches. Participants described a spectrum of desired family involvement, reinforcing that interventions should be flexible rather than prescriptive about family participation.

Garcia et al.'s (2025) evaluation of a bilingual, bicultural patient navigation program for Hispanic children with traumatic brain injury identified critical implementation factors: bilingual and literacy-appropriate materials and navigators who understood cultural norms facilitated success, while program duration limits (3 months), restrictive policies, funding gaps, and families' immigration concerns created barriers. This study uniquely examined organizational-level requirements, revealing that successful culturally adapted programs require systemic support beyond individual-level adaptations.

Family Dynamics and Health Literacy Gaps

Two qualitative studies examined family-mediated health management in dementia caregiving and diabetes prevention, revealing parallel patterns. Estrada et al. (2025) found Latino dementia caregivers (n=19) experienced profound isolation when families didn't share responsibilities, contradicting cultural expectations of collective caregiving. Gonzalez et al. (2025) found Hispanic men with prediabetes (n=15) emphasized personal agency for lifestyle change yet noted work environments and cultural food preferences (rice, beans, tortillas) created substantial barriers, revealing tension between individual responsibility beliefs and structural constraints.

Both studies revealed knowledge gaps: caregivers engaged in "trial and error" healthcare navigation due to limited dementia education; men expressed surprise at prediabetes diagnosis despite family history. These parallel findings suggest health literacy interventions addressing family health management must acknowledge both the complexity of family dynamics (not assuming uniform support) and the structural barriers that limit individual agency.

Within-Group Heterogeneity and Nativity Differences

Two large-scale studies demonstrated critical within-group variation. Sánchez-Johnsen et al. (2025) found Mexican women differed significantly from Puerto Rican women on multiple characteristics: age (38.8 vs 44.0 years, p=.003), education (17.3% vs 3.2% less than 9th grade), language preference (49% vs 81.1% English, p<.001), weight status (31.7% vs 18.9% normal weight), smoking (7.7% vs 17.9%), and insurance (50% vs 76.8%). These findings demonstrate health interventions cannot treat "Hispanic" as one category, language preferences, acculturation, obesity rates, and health behaviors vary substantially by origin group.

Odame et al.'s (2025) study of 1,126 adults revealed discrimination experiences differ fundamentally by nativity: US-born individuals reported significantly more discrimination than foreign-born (43.20% vs 31.94%, p<.001). When stratified by birthplace, anxiety and depression were associated with discrimination only among US-born individuals (RRR=3.69), while loneliness was associated with discrimination for both US-born and foreign-born groups. Villamil Grest et al.'s (2025) longitudinal study of 1,179 youth found those experiencing combined psychological abuse and discrimination showed significantly higher alcohol use (B=.316, p=.04) than those experiencing microaggressions alone, providing longitudinal evidence that discrimination's effects accumulate over developmental periods.

Results and Discussion

This review demonstrates that Hispanic cultural origins shape health behaviors through complex pathways involving familismo, language, acculturation, health literacy, cultural beliefs, and discrimination. Cultural factors operate at multiple levels simultaneously, requiring multi-level intervention approaches.

Familismo functions context-dependently, most protective when aligned with individual preferences and actual family capacity, most harmful when expectations exceed reality. Language variation within Hispanic populations challenges monolithic approaches. Health literacy emerges as a powerful mediator (62% of variance), suggesting it should be a priority intervention target. Additionally, greater acculturation (US birth, higher generation) increases discrimination exposure, with mental health symptoms predicting discrimination only among US-born individuals. These patterns suggest interventions must be tailored by nativity status, language preference, and acculturation level rather than treating "Hispanic" as homogeneous.

Methodological Strengths and Limitations

The ten studies demonstrated diverse methodologies, each contributing unique evidence while presenting distinct limitations. The strongest evidence came from large-scale studies using rigorous statistical approaches: Odame et al.'s (2025) cross-sectional survey (n=1,126) used polytomous logistic regression with birthplace stratification; Villamil Grest et al.'s (2025) longitudinal design (n=1,179 across three timepoints) used latent class analysis; Tailakh et al.'s (2025) path analysis (n=149) established clear directional relationships.

Qualitative studies provided essential depth but faced sample size limitations. Four studies (n=12-19) used rigorous frameworks to identify themes, yet small samples limit generalizability. However, convergence of findings across multiple small studies strengthens confidence in patterns like familismo's dual nature. Pilot intervention studies (n=10-21) demonstrated feasibility and exceptional retention (90.5-100%), suggesting cultural adaptation enhances engagement, yet pre-post designs cannot establish causality.

Critical methodological gaps emerged: only one longitudinal study limits causal inference; zero RCTs (randomized control trials) means no causal evidence for interventions; small pilot samples provide insufficient statistical power; geographic concentration limits generalizability; overrepresentation of Mexican/Puerto Rican populations constrains applicability; self-report measures introduce bias; COVID-19 timing may limit generalizability. However, methodological diversity serves the field well, qualitative studies generate hypotheses that quantitative studies test at scale, while pilot interventions demonstrate feasibility before costly RCTs. The field now needs rigorous experimental designs to establish causality and intervention effectiveness.

Implications for Practice

Effective health interventions for Hispanic populations must recognize heterogeneity within this diverse group, assessing rather than assuming language preferences and cultural values. Health literacy emerges as a critical intervention target, particularly through bilingual education. Interventions should address both individual factors (knowledge, self-efficacy) and structural barriers (work environments, healthcare access, discrimination) simultaneously, recognizing personal agency alone cannot overcome systemic constraints.

Practitioners must thoughtfully navigate familismo's dual nature as both resource and stressor, assessing individual family dynamics rather than prescribing uniform involvement. Variation across contexts suggests interventions should offer flexible family participation options aligned with actual family capacity. Birthplace and acculturation status should inform mental health intervention design, given US-born individuals experience significantly more discrimination. Community-engaged approaches using advisory boards ensure cultural relevance and enhance retention, as demonstrated by exceptional completion rates (90-100%) in culturally adapted physical activity interventions.

Limitations

This review's findings are constrained by predominantly cross-sectional designs (preventing causal inference), small sample sizes in qualitative and pilot studies (limiting statistical power and generalizability), geographic concentration in the Southwest and select urban areas, overrepresentation of Mexican and Puerto Rican populations with minimal inclusion of other Hispanic subgroups, reliance on self-report measures (introducing potential bias), and COVID-19 pandemic timing for several studies (potentially limiting generalizability). Additionally, few studies examined intersectionality or how cultural factors interact with social identities beyond gender and birthplace.

Future Directions

Given identified methodological gaps, future research should prioritize specific study designs and populations. First, randomized controlled trials (n>200 per arm) should compare culturally adapted interventions to standard care, testing whether cultural adaptation produces superior retention and outcomes and whether addressing health literacy and cultural factors together produces synergistic effects.

Second, longitudinal cohort studies following diverse Hispanic subgroups across decades could illuminate how cultural factors and health behaviors change across generations. Following first-, second-, and third-generation immigrants from adolescence through older adulthood would clarify whether paradoxical acculturation effects persist, intensify, or diminish. Such studies should oversample underrepresented groups (Cuban, Dominican, Central American, South American) and include biomarkers alongside self-report. Third, mediation and moderation studies using structural equation modeling could test specific pathways linking cultural factors to health. Such studies require samples of n>500 and repeated measurement to establish temporal precedence. Fourth, multi-level intervention trials testing simultaneous individual, family, community, and policy-level changes could address the multiple levels at which cultural factors operate. Fifth, intersectionality research examining how cultural factors interact with gender, sexual orientation, disability status, and socioeconomic position remains critically needed. Finally, studies should examine protective cultural factors and resilience alongside risks, informing asset-based rather than deficit-based interventions.

Specific high-priority studies include: (1) a multi-site RCT (n=400) testing culturally adapted diabetes prevention in underrepresented populations; (2) a 20-year cohort study (n=2,000) following Hispanic youth across generations to map acculturation trajectories; (3) an implementation science trial testing real-world delivery through Federally Qualified Health Centers; and (4) ecological momentary assessment studies capturing real-time experiences of discrimination, acculturative stress, and health behaviors.

Conclusion

This review demonstrates that Hispanic cultural origins shape health behaviors through complex, multifaceted pathways. Effective health interventions must recognize within-group heterogeneity, prioritize health literacy as a key mediator, address both individual and structural factors, and thoughtfully navigate the dual nature of cultural factors as both protective resources and potential stressors.

Among methodologies reviewed, longitudinal designs with large samples (Villamil Grest et al., n=1,179; Odame et al., n=1,126) and path analysis with adequate statistical power (Tailakh et al., n=149) produce the best quality evidence in terms of reproducibility and error rates. These studies provide clear effect sizes, confidence intervals, and statistical controls that enable replication. In contrast, small qualitative studies (n=10-19) and pilot interventions without control groups, while providing valuable preliminary insights, have lower reproducibility due to small samples and lack of randomization.

The critical gap is the complete absence of randomized controlled trials, no study establishes causal relationships or intervention efficacy through experimental design. Additionally, underrepresentation of Cuban, Dominican, Central American, and South American populations (appearing in only 1-2 studies each) limits generalizability across Hispanic subgroups.

Given these quality assessments and gaps, next research steps should prioritize: (1) adequately powered RCTs (n>200 per arm) testing culturally adapted interventions against standard care to establish causality; (2) longitudinal cohorts (n>2,000) following diverse Hispanic subgroups across generations to clarify acculturation trajectories; and (3) replication studies confirming key findings (particularly health literacy mediation and familismo's dual nature) across different Hispanic populations and geographic regions.

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Claude was used for help with grammar check/cleaning up and structure. I asked Claude, "Claude, can you please look over my paper and let me know if there are any words I may have misspelled or if any sentences need enhancing and where". I also asked Claude, "Claude can you look over my paper and let me know if the structure flows, if there's a section that doesn't, what do you recommend I do?". MyBib was used for keeping track of my sources and helping format my citations, Google docs was used to draft this first, second, and final draft. Canva was used to create figures 1 and 4, Jupyter Notebook (Python) was used to create figures 2 and 3.